

Joint Commission Accreditation Readiness Guide

Sunrise Medical Center

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Department: Quality and Accreditation

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Distribution: All Sunrise Medical Center Staff

1. Purpose

Sunrise Medical Center maintains accreditation through The Joint Commission (TJC), which requires continuous, not just periodic, compliance with the National Patient Safety Goals and hospital accreditation standards. This guide summarizes staff-facing expectations so that every department is survey-ready at all times, since TJC surveys are unannounced.

2. National Patient Safety Goals Summary

- Use at least two patient identifiers (name and date of birth) before any procedure, medication administration, or specimen collection.
- Improve the effectiveness of communication among caregivers, including read-back verification of verbal and telephone orders.
- Reduce the likelihood of patient harm associated with the use of anticoagulant therapy through standardized dosing protocols.
- Maintain and communicate accurate patient medication information across every transition of care (medication reconciliation).
- Reduce the risk of healthcare-associated infections through hand hygiene compliance and central line/catheter bundle adherence.
- Identify patients at risk for suicide through standardized screening in emergency and behavioral health settings.

3. Universal Protocol for Surgery

All surgical and invasive procedures require a documented pre-procedure verification, site marking by the responsible provider, and a formal time-out immediately before starting the procedure, during which the entire team confirms patient identity, procedure, and site aloud before any incision or instrumentation begins.

4. Environment of Care Requirements

Every department is responsible for maintaining clear egress pathways, properly stored and labeled hazardous materials, functioning fire extinguishers with current inspection tags, and unobstructed access to electrical panels and fire suppression equipment at all times. Surveyors routinely walk clinical and

non-clinical areas alike, including supply closets and staff break rooms.

5. Staff Interview Readiness

Any staff member may be interviewed directly by a surveyor during tracer activities and must be able to describe, in their own words, how they would respond to a fire, where the nearest fire extinguisher and pull station are located, how to report a safety event, and how to identify a patient using two identifiers. Staff are encouraged to ask surveyors for clarification rather than guess at an answer.

6. Document and Record Availability

Departments must be able to produce current competency records, equipment maintenance logs, and departmental policies within minutes of a surveyor request. Outdated policy binders or missing sign-off sheets are among the most common findings cited during mock surveys and must be corrected immediately upon discovery.

7. Mock Survey Program

The Quality and Accreditation Department conducts unannounced mock tracer surveys quarterly across all clinical units. Findings are shared with unit leadership within 48 hours, and corrective action plans with named owners and due dates are required for any finding rated moderate or higher risk.

8. Roles and Responsibilities

Role	Responsibility
Unit Manager	Ensure staff competency documentation and unit readiness
Quality Department	Coordinate mock surveys, track corrective action plans
All Staff	Maintain working knowledge of safety procedures and be interview-ready
Executive Sponsor	Report accreditation status to the Board Quality Committee quarterly

9. Related Documents

SMC-QI-002 National Patient Safety Goals Compliance Checklist; SMC-FAC-010 Environment of Care Rounding Procedure; SMC-QI-014 Mock Tracer Survey Program Guide.

Appendix A: Frequently Asked Questions

Q: What should staff do if a surveyor asks a question they don't know the answer to? A: It is acceptable and expected to say 'I don't know, but I know where to find that information' and locate the correct policy or a supervisor rather than guessing.

Q: How far in advance is a Joint Commission survey announced? A: TJC surveys are unannounced; the survey window is only known to fall within the accreditation cycle, which is why continuous readiness rather than pre-survey preparation is required.

Q: Are contracted and agency staff included in accreditation readiness expectations? A: Yes, all individuals providing care under the hospital's license, including travel nurses and locum physicians, are expected to

meet the same readiness standards as permanent staff.

Appendix B: Common Scenarios

Scenario — Surveyor finds an expired fire extinguisher inspection tag: This is documented as a finding regardless of whether the extinguisher itself is functional; inspection tags must be current at all times and checked monthly by unit staff.

Scenario — Surveyor conducts a tracer interview with a housekeeping staff member: Environmental Services staff are expected to know basic safety data sheet locations, how to respond to a spill, and who to notify for a safety concern, even though they are non-clinical staff.

Revision History

Version	Date	Description
1.0	Aug 1, 2020	Initial release
1.4	May 12, 2025	Updated NPSG list for current cycle